Insanity in the 18th and 19th centuries

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John Conolly: a reconsideration

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John Conolly is unquestionably the most famous of the first generation of British alienists, and in the hagiography that once passed for psychiatric history, his accomplishments were regularly lauded. 'It is to Conolly', said Benjamin Ward-Richardson[1], 'that we really owe the modern humane treatment of the insane as it exists today in all its beneficent ramifications . . . [he] has placed us first among all the nations as physicians of mental disease.' Widely viewed as the father of non-restraint, his labours brought him relatively modest financial rewards, but secured him a prominent place in the pantheon of psychiatric heroes.

Conolly's connections with Stratford-upon-Avon were multiple, and extended from the very beginning to the very end of his professional life. After two abortive efforts to establish a general practice in Sussex, he moved his growing family to Stratford in the spring of 1823. Recently graduated from the University of Edinburgh (MD, 1821), he lacked the means to buy into an established practice and had few connections with metropolitan medical elites, complicating his efforts to earn even a modest competence as a general practitioner in a period when professional competition was at its height[2].

Like any aspiring professional, Conolly needed to make himself professionally and socially visible. He quickly joined the town council, took the lead in establishing a dispensary for the poor, and helped form the Shakespearean Club (an indication of a literary interest that bound him to Stratford till his death). In September 1825, he became mayor, at an annual salary of just over £100, perhaps a third of his income at this time, and he also served as inspecting physician to the Warwickshire madhouses, a post which required only an occasional visit to a half-dozen local establishments. Insanity, the subject of Conolly's MD dissertation, was also the theme of his first important publication[3], but to grasp the peculiarities of his career, one must bear in mind that this tenuous connection with the local 'trade in lunacy' was his sole clinical connection with alienism until he was past the age of 40.

Nothing as yet suggested that Conolly would become anything more than an obscure and modestly successful provincial practitioner - someone who might today form anonymous grist for a sociological study of the exigencies of early 19th century medical practice, but an unlikely candidate for a prominent place in the gallery of heroes of English psychiatric history. Moreover, when the chance unexpectedly *did* come to secure a measure of professional prominence, it was in medicine rather than in psychiatry, and Conolly fumbled it so badly that he apparently sealed his fate.

The foundation of the new University of London, in 1827, included a medical school. New and inadequately capitalized, it initially offered only a precarious future and limited salaries to its faculty, who were frequently young or relatively unknown. Still, an appointment presented a real prospect of crossing that great divide separating the social and financial world of elite London physicians from the humble surroundings of the rest of the profession, and Conolly was both flattered and delighted when his Edinburgh connections secured one for him.

Dismayingly, only two years later, he confronted professional and financial ruin. To an extent, this was hardly his fault. The first Warden, Leonard Horner, proved a disastrous choice, and the ill effects of declining enrolments and salary cuts on staff morale were compounded by the constant interference of the proprietors. But while others survived and even flourished despite these difficulties, Conolly was soon drowning in debt and quarrelling ever more openly with his employers. Worse yet, his attempts to augment his income from private practice were thoroughly unsuccessful. Unsurprisingly, he resigned in December 1830, and removed his wife and four children back to Stratford. After the merest glimpse of success, he was thrust back into provincial obscurity.

His tribulations continued. The attempt to pick up the threads of his old practice proved a dismal failure, and soon he felt compelled to move to Warwick. Throughout what was to be an 8-year exile in the provinces, Conolly's financial position remained difficult, in part because of the need to satisfy his London creditors. Soon, he was close to despair. As he wrote to one of his few remaining metropolitan contacts[4], were he to visit, 'You will find me a very rustic physician with some provincial fame, no doubt, but as my foolish friends say, *buried*'. Revealingly, he continued: 'I often wish I really were . . . The London University has provided me for life with incurable care'.

Finally, in desperation, and 'not much encouraged thereto by his friends, who regarded such a step as the suicide of reputation and the confession of complete failure in life'[5], he applied for the superintendency of the Middlesex County Lunatic Asylum at Hanwell. At least this offered the security of a salary of £500, and free room and board. To his dismay, however, his application was rejected, apparently because of his liberal politics. Conolly's humiliation was complete. Facing an even blacker future, he once more uprooted his household and removed to Birmingham, to see whether his luck would change. At 42, this latest failure appeared to have permanently dashed all his hopes of worldly success.

Ironically enough, he was rescued from this depressing prospect by someone else's failures. The

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superintendency at Hanwell had originally fallen vacant when the Middlesex magistrates decided to experiment with a system of divided authority, allowing the superintendent to continue as the final arbiter of medical matters, but handing over administrative chores to a lay steward. The arrangement proved unworkable, and conditions in the asylum degenerated until they verged on anarchy. The magistrates were finally forced to intervene, and when the job fell vacant for a second time, Connolly's application was successful.

Against all reasonable expectation, a man incapable of managing his own affairs with even modest competence turned out to be an able and effective administrator of the largest and most visible English asylum. Within weeks, the magistrates cheerfully announced that conditions were remarkably improved. Conolly had at last found something he could do well. His achievements, most notably his introduction of a policy of total non-restraint, brought him, in rapid succession, national recognition, royal notice and favour, election to a fellowship of The Royal College of Physicians and, ultimately, recognition as 'the most valuable consulting physician in mental disorders in Great Britain, and I suppose, in the world'[6].

But Conolly's tenure at Hanwell was extraordinarily brief: barely 4 years after he became superintendent, the magistrates revived the experiment with a lay administrator. Conolly would have none of it. Complaining of 'the absurdity - I could almost say the criminality - of committing one of the most serious of human maladies to the charge of anyone uninstructed in medicine'[7], he promptly resigned. Henceforth, his livelihood depended upon moderately rewarding practice as a private consultant, the proprietorship of a series of private asylums, and occasional, sometimes embarrassing forays into the courtroom, where he proved an ineffectual expert witness in both civil and criminal trials.

The evolution of Conolly's ideas on insanity bears an uncannily close relationship to these twists and turns in his private fortunes. Moreover, his views turn out to be far more complex and even contradictory than can be encompassed in any simple portrait of him as 'the father of non-restraint'. His first book on insanity[3], appearing in the midst of the campaign for lunacy reform, is in many respects a very conventional treatise. But in one vital particular, it dissents sharply from the contemporary orthodoxy. English lunacy reformers were absolutely convinced that asylum care was the only appropriate form of treatment for the insane. Conolly emphatically rejected such notions. There were, he conceded, certain classes of patients for whom public asylums were 'unavoidable evils', but this regrettable necessity should not obscure the fact that 'it is a far different place for two thirds of those who are confined there . . . to all these patients confinement is the very reverse of beneficial. It fixes and renders permanent what might have passed away and ripens eccentricity or temporary excitement or depression, into actual insanity'.

The defect, as Conolly saw it, was structural and thus irremediable. Confinement in an institution acted like a self-fulfilling prophecy, intensifying and even creating the very behaviours that were its alleged justification. For whole classes of lunatics, therefore - and these included all recent and curable cases, the depressed, the convalescent, those suffering from mere weakness of intellect, and the non-dangerous chronically insane - asylum treatment was grossly inappropriate.

If social practices could be brought to reflect these realities, 'the patients *out* of the asylum being the majority, and consisting of all whose circumstances would ensure them proper attendance - better arrangements might be made for the smaller number of public asylums, or central houses of reception'. Such asylums must be state funded, to avoid the problems created by a system in which 'the patients are transmitted, like stock-in-trade, from one member of a family to another' - a free trade in lunacy which attracted, besides a handful of 'respectable, well-educated, and humane individuals', the 'ignorant and ill-educated' and those 'capable of no feeling but a desire for wealth'. Each asylum should then become a centre in which aspiring physicians were taught to recognize and treat insanity, for this would give the average medical practitioner both the competence and the confidence to treat most cases of insanity on a domiciliary basis.

Here was a powerful critique of the developing conventional wisdom. But if Conolly hoped it would serve to advance his reputation and enlarge his private practice, he was soon disabused. For the most part, his suggestions were simply ignored.

A decade later, placed in charge of the country's largest public asylum, Conolly made an abrupt volte-face and embraced institutional treatment. It was the proud boast of the reformers that the adoption of the new system of moral treatment pioneered by the Tukes at the York Retreat did away with the cruelties previously visited upon the insane. Whips and chains, those traditional accoutrements of the madhouse, were, like the straw and stench that were their inevitable accompaniments, to be banished from modern practice. Still, even the most sanguine did not 'anticipate that the most enlightened and ingenious humanity will ever be able entirely to supersede the necessity of personal restraint'[8].

Yet this was precisely Conolly's accomplishment. Beginning with his first report at Hanwell, he boldly asserted that 'the management of a large asylum is not only practicable without the application of bodily coercion to the patient, but that, after the total disuse of such a method of control, the whole character of the asylum undergoes a gradual and beneficial change'. Rather than a regrettable necessity, or even a means of cure (as its defenders alleged), restraint 'was in fact creative of many of the outrages and disorders to repress which its application was commonly deemed indispensable . . .' so that 'restraints and neglect may be considered as synonymous'[9].

These were large and astonishing claims, greeted in many quarters with scepticism, if not outright hostility. They were, sniffed 'Medicus' in the correspondence columns of *The Times*, 'a piece of contemptible quackery and a mere bait for the public ear'[10]. But while many medical men reacted with extreme suspicion, elsewhere the new system attracted powerful support. In swift succession, Thomas Wakley, editor of the *Lancet*; Samuel Tuke, author of the English version of moral treatment; Anthony Astley Cooper, leader of the parliamentary forces seeking lunacy reform; and *The Times* itself hastened to endorse Conolly's efforts. Royal patronage was not far behind, and then a host of professional honours:

chairing the third annual meeting of the alienists' association, election as FRCP; a DCL from Oxford; the presentation of bust, plate and portrait to commemorate his accomplishments; and the adoption of non-restraint as the ruling orthodoxy of British asylumdom.

It is curious that Conolly alone received the major credit for non-restraint. Priority disputes are, of course, endemic in science and medicine. In this instance, he had a rival who had every reason to feel aggrieved when Conolly's powerful friends, while conceding that he 'no doubt received important assistance from fellow labourers in the same field', closed ranks around the proposition that 'Dr Conolly himself put an end to the use of all forms of mechanical restraint in our asylums'[11]. For such claims, as Conolly himself periodically acknowledged, were at best a serious distortion. Non-restraint was introduced, not by him but by Robert Gardiner Hill, then a 24-year-old house surgeon at the provincial asylum at Lincoln[12]. It was, in fact, a visit to Lincoln that prompted Conolly to try the new system.

Yet Hill's obvious claim to priority brought him little honour and scant reward of any other sort. Though bearing the brunt of the early assaults on the system as speculative and wildly misguided, he was granted none of the subsequent recognition and social lionization so readily accorded to Conolly. On the contrary, machinations among the staff and governors at the Lincoln Asylum forced his resignation there, and he was unable to obtain another asylum post.

Nor was Conolly's role in this precisely innocent. With whatever motives, he consistently declined to give Hill his due. That he had borrowed non-restraint from Lincoln he could not deny; that the discovery was Hill's he constantly sought to obscure. And when Hill in exasperation at length lashed out at his now-deceased rival, he succeeded only in further tarnishing his reputation. His shrill and strident claims of priority, his wearisome marshalling of minutiae were, as Alexander Walk remarked, 'not only boring, but repellent'[13]. As he proved chronically unable to grasp, one who exhibited such boorish and ungentlemanly qualities could never hope to be accorded a place of honour in a profession desperate to dissociate itself from all that smacked of lower class, tradesman-like behaviour.

By contrast, the elegant and socially graceful Conolly inflicted no such handicaps on himself. Despite a patient population nearing 1000 crammed into buildings originally designed for half that number, and notwithstanding a dismally low cure rate, his asylum was widely regarded as a splendid advertisement for the merits of reform and non-restraint. And though he occasionally uttered mild complaints that the asylum was too big, the man who had formerly been such a stern critic of the anti-therapeutic properties of asylum life, now embarked on a campaign to persuade others of the advantages, indeed the necessity of expanding the number of county asylums.

Ironically, just as the campaign got under way, Conolly's resignation at Hanwell severed his formal links with the new system of public asylums. However principled, his departure once more confronted him with the difficult task of securing a livelihood, a problem rendered the more pressing by the fecklessness of his son (married with 7 or 8 children, and no secure occupation), and by the failure of his daughters to land suitable husbands. 'A man', said Conolly a few years later, 'must live by his profession, and a physician who devotes himself to mental disorders has to deal with a very small portion of population, and he generally adds to his consulting practice, the plan of having a place where the treatment of patients can be conducted entirely under his own observation'[14]. What this euphemistic phraseology scarcely disguised was that Conolly was now forced to enter the trade in lunacy. This was particularly difficult for him, since he had long been one of the most vocal and forceful advocates of the position that 'every lunatic asylum should be the property of the State'[3]. With its obvious overtones of trade and its long-established unsavoury reputation (to which his own earlier writings had in no small measure contributed), the business of running a private asylum was widely regarded as one of the most *declassé* forms of medical practice. Repugnant or not, it was unavoidable, and before his death Conolly used the occasion of his second Presidential Address to the Medico-Psychological Association to deliver a Panglossian portrait of private asylums - prompting Charles Reade to satirize him, in his best-selling melodrama, *Very Hard Cash*, as the very epitome of the intellectual bankruptcy and moral obtuseness of the psychiatric profession[15].

It is possible, if one is charitably inclined, to regard the evolution of Conolly's views as the product of greater experience and maturity. In his earliest years, he was disposed to promote impractical, if superficially attractive, schemes of non-asylum treatment. Later acquaintance with the realities of treating mental disorder prompted him to revise his ideas, as did the experience of running a private asylum.

Alternatively, his gyrations can be accorded a more cynical interpretation. As Conolly once remarked, 'When men's interests depend upon an opinion, it is too much to expect that opinion always to be cautiously formed, or even in all cases honestly given'[3]. Some of his contemporaries saw the parallels between the evolution of his ideas and the unfolding of his career as more than mere coincidence: Bucknill, whose own intellectual development was in precisely the opposite direction, from an enthusiastic advocate to a scathing critic of the asylum, was convinced that Conolly's judgment had simply been subverted by self-interest, voicing regret that 'advancing years and personal interests had made [him] indulgent to the evils he had denounced'[16].

My own preference is for a less moralistic perspective. It is instructive how difficult it is for modern readers to portray Conolly's intellectual journey as progress. Large segments of informed opinion have now come to view the asylum as an almost unmitigated disaster, a fatally mistaken approach to the problems of managing the mentally ill, and one that cannot be too swiftly consigned to the dustbin of history. From this perspective (which itself warrants close scrutiny)[17], Conolly's views seem to mark an almost perverse shift *from* enlightenment and *to* error. It is to his earliest work that our contemporaries turn when they count him the author of 'principles of treatment that have scarecely been improved in all the succeeding epochs of vanguard practice'[18]. For Victorians, however, it was precisely this early critique of the asylum and advocacy of domiciliary care that was anomalous. The abandonment of such

aberrant opinions in favour of an elaborate defence of asylum treatment required no special explanation: it simply represented an acknowledgment of the findings of modern medical science. This should only serve to remind us, here as elsewhere, just how slippery the concept of 'knowledge' is in the human sciences, and how profoundly dependent the content of that knowledge is upon the nature of the larger social order.

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The Victorian controversy about the insanity defence

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Provoked by defence counsel while summing-up at a trial for murder in 1856, the judge launched an attack on psychiatric witnesses:

'He certainly believed that the jury were as competent to form a correct opinion on matters of this kind as anybody else . . . Experts in madness! Mad doctors! Gentlemen, I will read you the evidence of these medical witnesses - these "experts in madness", - And if you can make sane evidence out of what they say, do so; but I confess it's more than I can do . . . If the theory of these gentlemen were true of the prisoner, it would be equally true in the case of every criminal, and form a conclusive reason for liberating every person charged with crime.'[1]

A few years later an Edinburgh medical professor responded to such humiliating attacks:

'Medicine says a man may be insane and irresponsible, and yet know right and wrong; law says a knowledge of right and wrong is the test of both soundness of mind and responsibility to the law. Medicine says, restrain and cure the insane and imbecile offender against the law; law says, hang, imprison, whip, hunger him, and treats medical art with contempt.'[2]

Whatever the rhetorical excess displayed in such outbursts, clearly something very substantial was at issue. Or rather, it is because an extraordinary range of issues - personal, professional, local, national - were at stake that such passages have so much emotional colour. I will concentrate here on only one theme: whether and in what sense the law represented *prejudice*, what critics saw as the persistence of mediaeval thought long past its time, while psychological medicine represented scientific and humanitarian *progress*. I will simplify in order to highlight particular factors in a very complex situation.

Victorian reform and 'the facts of nature'

To 20th century psychiatrists who argued successfully for the 1930 and 1959 Mental Treatment and Mental Health Acts, it was quite clear that progress depended on replacing legal by medical criteria in the administration of mental illness. Many of them would have found it difficult to sympathize with, and impossible to anticipate, the legalistic direction underlying aspects of the 1983 Act (*see* Unsworth[3] for an excellent study of legislation from 1890 to the present). But recent legislation suggests that we might look again at the medical reformers' criticisms of the criminal law in the 19th century. In what sense did 'experts in madness' think that progress depended on eliminating statements like the one quoted from Baron Bramwell in 1856? And why were legal men and, it must be acknowledged, many other professional people (including many doctors) opposed to the reformers' view of progress? And, more generally, might we even ask whether there are general criteria for deciding what is progress?

Early and mid-Victorian legislation was often preoccupied with 'reform': above all with extending the franchise, and with major revision of society's

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